

GULFSHORE VASCULAR & ENDOVASCULAR

412 Bayshore Boulevard, Suite 300 | Tampa, FL 33606 | (813) 555-0140 | Fax (813) 555-0141
NPI: 1497753102 | Tax ID: 59-5550142 | Fictional practice created for demonstration

PATIENT MEDICAL CHART

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1. Patient demographics

Patient name	Maria Elena Santos
Date of birth	June 12, 1961
Age at visit	64 years
Sex	Female
MRN	GSV-44190
Address	880 West Palm Avenue, Tampa, FL 33602
Phone	(813) 555-0174
Emergency contact	Luis Santos (spouse), (813) 555-0175
Primary care	Dr. Helen Cho, MD, (813) 555-0108
Preferred language	English and Spanish

2. Insurance

Primary insurance

Payer	Florida Blue, BlueOptions PPO
Member ID	XFB229184033
Group	FL-44021
Prior authorization	Required for varicose vein procedures
Auth telephone	1-800-555-0194

Secondary insurance. None on file.

3. Active problem list

ICD-10	Diagnosis	Onset	Status
I83.892	Varicose veins of left lower extremity with other complications	2024	Active
I87.2	Chronic venous insufficiency (peripheral)	2024	Active
I10	Essential hypertension	2016	Active
E66.9	Obesity, unspecified	2018	Active

4. Current medications

Medication	Dose	Frequency	Indication
Amlodipine	5 mg	Daily	Hypertension
Hydrochlorothiazide	12.5 mg	Daily	Hypertension

Medication	Dose	Frequency	Indication
Ibuprofen	400 mg	As needed	Leg aching

5. Allergies

Allergen	Reaction	Severity
No known drug allergies	Not applicable	Not applicable

6. Visit notes — vascular surgery

Visit date: August 20, 2025 | **Attending:** Elena Vasquez, MD, FACS | **Type:** New patient consult

Chief complaint. Left leg aching and heaviness after standing.

History of present illness. Maria Santos is a 64-year-old cashier who stands for eight-hour shifts. For fourteen months she has had aching and heaviness of the left calf that starts by mid-shift. She has cut two evening shifts a week because the pain significantly interferes with activities of daily living: she cannot finish a full cashier shift without sitting, and she stopped walking her dog. Elevation at night helps slightly. She has never worn compression stockings. She has no venous ulcer, no bleeding from a vein, and no history of superficial thrombophlebitis. She has no history of deep-vein thrombosis. She denies rest pain and denies calf pain that starts only with a fixed walking distance.

Examination.

Blood pressure 142/86 mmHg, right arm, seated	Heart rate 78 bpm
BMI 32.4	Left leg Medial calf varicosities. Gaiter hyperpigmentation. No ulcer. No edema at this visit.
CEAP C4a, symptomatic (pigmentation, no active ulcer)	Pulses Femoral, popliteal, and pedal pulses 2+ both legs

Assessment and plan.

Symptomatic left great saphenous disease, suspected. CEAP C4a. No ulcer, no bleeding, and no thrombophlebitis, so a compression trial is required before ablation.

Order bilateral venous duplex reflux study.

Fit 20-30 mmHg knee-high graduated compression stockings today, August 20, 2025. Wear them daily while upright. Return in six weeks.

Electronically signed: Elena Vasquez, MD, FACS — August 20, 2025 at 11:20 AM

Visit date: October 2, 2025 | **Attending:** Elena Vasquez, MD, FACS | **Type:** Follow-up

Chief complaint. Compression follow-up and duplex review.

History of present illness. She returns six weeks after the stockings were fitted. She wore the 20-30 mmHg knee-high stockings on workdays. The compression trial documented in this chart runs from August 20, 2025 through October 2, 2025, which is six weeks, not three months. Aching still starts by mid-shift and still significantly interferes with activities of daily living. There is still no ulcer, no bleeding, and no superficial thrombophlebitis.

Examination.

Stocking use Most workdays since August 20, 2025	Left leg Varicosities unchanged. Pigmentation unchanged. No ulcer.
Edema Trace ankle edema at the end of the day, per patient	Right leg No symptomatic varicosities

Assessment and plan.

Duplex of August 28, 2025 shows left great saphenous reflux and a vein diameter over 3 millimeters. CEAP remains C4a.

Indication for treatment is persistent pain and swelling that interfere with work. Ulcer, recurrent thrombophlebitis, and bleeding are absent.

The compression trial is six weeks. A three-month trial is not documented.

She asks to schedule ablation. Order endovenous radiofrequency ablation of the left great saphenous vein.

Electronically signed: Elena Vasquez, MD, FACS — October 2, 2025 at 2:05 PM

7. Diagnostic reports

Venous duplex reflux study — August 28, 2025

Ordering physician	Elena Vasquez, MD, FACS
Facility	Gulfshore Vascular Lab
Indication	Left leg aching, suspected venous insufficiency
Technique	Upright duplex with compression maneuvers

Interpretation. Left great saphenous vein: reflux lasting 1.4 seconds at the saphenofemoral junction. Great saphenous diameter 6.2 millimeters in the proximal thigh. Small saphenous vein is competent. Deep veins are patent, with no deep-vein thrombosis. No incompetent perforator measures 3.5 millimeters or larger. Right great saphenous vein is competent. Impression: left great saphenous reflux with a vein diameter of 6.2 millimeters. CEAP clinical class C4a is recorded from the referring examination.

Electronically signed: James Whitfield, MD, RPVI — August 28, 2025

8. Planned procedure

Planned procedure	Endovenous radiofrequency ablation of the left great saphenous vein
CPT	36475 — radiofrequency ablation, first vein
Laterality	Left leg
Vein	Great saphenous vein only. No perforator is included.
Surgeon	Elena Vasquez, MD, FACS
Facility	Gulfshore Vascular & Endovascular
Payer	Florida Blue BlueOptions PPO
Authorization status	Not submitted. Packet requested October 2, 2025.

Clinical indication summary

Diagnosis: left great saphenous reflux with chronic venous insufficiency (I83.892, I87.2).
Duplex August 28, 2025: reflux 1.4 seconds, diameter 6.2 millimeters, left great saphenous vein.
CEAP C4a, symptomatic. No active or healed ulcer.
Symptoms significantly interfere with activities of daily living (cashier shifts).
Compression: 20-30 mmHg stockings from August 20, 2025 through October 2, 2025 (six weeks). Symptoms have not improved.
A three-month compression trial is not in the chart.
No ulceration, no bleeding varicosity, and no superficial thrombophlebitis.

This document is for clinical and administrative demonstration only. Patient identifiers are fictional.